8 Psychosocial Support Without Coercion: Consequences, Dilemmas and Possible Ways Out

8.1 Introduction

Although the current debate about the abolition of coercion through the UN Convention on the Rights of Persons with Disabilities continues to make waves, the demand is by no means new, even in professional circles. As early as 1999, the US Association of State Mental Health Programme Managers published a position paper that, while justifying the use of restraint in certain circumstances, formulated the goal of "... preventing, reducing and ultimately eliminating the use of seclusion and restraint..." [1]. This demand was met by many professional associations in the following years [2]. Interestingly, these demands were virtually uncontroversial at the time. This was possibly because the main focus was on reduction and not elimination.

The dispute began with the demand for the abolition of coercion. The possible abandonment of coercion in what we usually refer to today as mental health care was already described in the second chapter, with quotations from psychiatric professionals as quasi "inhumane" and a "historical step backwards". According to the conventional psychiatric interpretation, coercion protects people with mental disorders primarily in the context of endangering others and themselves.

From an empirical perspective, this connection between coercion and danger to oneself and others is not as compelling as it appears to be at first glance. With the abolition of psychiatric hospitals in Italy, there is, to a certain extent, a natural experiment [3]. It is well-known that inpatient bed capacity has significantly decreased in Italy since the end of the 1970s. However, unlike in many countries, this has not led to an increase in compulsory admissions; in contrast, it has actually led to a significant decrease. The same applies to forensic hospitalisation. Furthermore, the national suicide rate has not increased. Although coercion is still possible in psychiatric settings, developments have shown that a changed support system that involves less coercion does not automatically create "communicating pipes" that shift restrictive practices.

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Nevertheless, a corresponding implementation of the UN CRPD would have considerable consequences, as it aims to completely abolish coercion in psychiatry. The American psychiatrist Paul Appelbaum, one of the most vehement critics of the UN CRPD, describes it with reference to criminal proceedings, saying that this would lead to convictions and prison sentences for people who were previously not considered responsible for their actions [4]. Many of those affected who are treated against their will in forensic hospitals today would therefore end up in prison, often without psychosocial support, as we know from the conditions in these institutions.

Even if I basically disagree with Appelbaum's fundamental opposition to the UN CRPD, in my opinion, he rightly criticises the fact that the proponents of the Convention remain silent about this issue. It concerns not only external endangerment and criminal behaviour, which are rarely mentioned but also self-harming and suicidal behaviour [5]. In my opinion, the advocates of the Convention are not doing themselves or the cause any favours by doing this, because they are allowing psychiatric critics a considerable and not easily refutable argument. If serious crimes were committed by people with psychosocial problems who could have been treated against their will, the public and the media would probably show no sympathy for this. The same applies, for example, to a possible suicide of a young adult who was in contact with psychiatric services but could not be held there involuntarily for reasons of the UN CRPD.

In the discussion to date about the abolition of coercion in psychiatry, the proponents therefore failed to analyse the obvious consequences and problems that such a step would entail. What consequences would this have for forensic psychiatry? How do we deal with people who want to take their own lives? How do we deal with people who are suffering from dementia? It is not only at first glance that the call for the abolition of coercion poses considerable dilemmas. If the abolition of coercion has a chance, these dilemmas and the associated difficulties must be addressed and analysed. This will be done below, at least to some extent. The following aspects will be analysed in detail: (1) the possible consequences of a preference-based declaration of being ill/not ill, (2) the possible consequences of preference-based psychosocial support, (3) the dilemmas associated with the removal of coercion in cases of aggression to others, and (4) the dilemmas associated with the renunciation of coercion in connection with suicidal tendencies and self-harm.

8.2 Consequences of a Preference-Oriented Self-Declaration of Ill/Not Ill

As a reminder, the justification for preference-based self-declaration was based on the empirical finding that an exclusive external declaration and diagnosis of "mental disorder" is based on insufficiently valid and sometimes arbitrary criteria. Nevertheless, the concept of a "mental illness" is widespread in modern society and is accepted by many people. However, there are also people who do not recognise this concept in general or for themselves in certain situations. This led to the conclusion that people should be able to decide for themselves whether they categorise

themselves as "mentally ill" or "not ill" and therefore whether to make use of support services.

A corresponding self-declaration leads to various problems given the current conditions in the healthcare system and in the judiciary. Let us stay with the healthcare system first. The current situation is that medical, therapeutic, psychological, (usually also) nursing and (possibly also) social work support services are linked to a medical diagnosis. Without a diagnosis, i.e. without the identification of an illness, no healthcare services are financially covered by social insurance or other agencies. This even applies in the rehabilitation system, for example, with the rehabilitation classification ICF (*International Classification of Functioning, Disability and Health*), which must be assessed in conjunction with the ICD disease classification and which presupposes a health problem as a disease or disorder. Without the identification of a corresponding health problem, the ICF may not actually be used [6].

If the external identification of a mental illness was to be cancelled, the person would still be left to make the diagnosis and support services still be used. In this case, an expansion of services in certain areas is certainly to be expected. There would be cases in which people would want to make use of support that is currently not possible without a diagnosis. Apart from the question of the extent to which such support services would be offered and financed at all, the question arises as to whether this could generally lead to an expansion of services and an associated increase in costs. In my opinion, this is not certain. It is highly likely that support services that currently require a visit to a medical specialist would be less utilised and, thus, be reduced in the future. The costs of medicalising certain social problems would also be reduced.

The consequences for the judicial system would be serious. In court proceedings, it would be up to the accused person to determine whether they had carried out a certain act as part of a "mental illness". Should they refuse to do so, they would be subjected to regular proceedings without any diminished capacity diagnosed by others coming into play. In contrast to many myths surrounding diminished capacity, this does not lead to shorter sentences or quicker release [7]. As these people are commonly detained in forensic hospitals, where the placements are usually not scheduled in advance, this often leads to significantly longer sentences. This is not the only reason why many of those affected do not agree with the finding of diminished culpability.

As already mentioned, a major dilemma arises from the often inadequate psychosocial support in prisons. Psychosocial problems do not necessarily have to have been present before the start of the sentence; imprisonment itself can also trigger them. Suicides are known to occur significantly more frequently in prison populations than in the general population [8]. These effects should be seen in particular against the background of the unattainable goal of prison sentences. These sentences have virtually no effect on crime, as older [9] and more recent [10] empirical studies have repeatedly found. Their sole purpose is to keep people away from other people for a certain period of time, but they do not serve their supposed purpose of deterring the punished people from committing further offences.

If the person declares themselves to be "mentally ill", has committed an offence within the framework of diminished culpability and is willing to undergo treatment, then they would, to a certain extent, be undergoing voluntary psychiatric forensic treatment, which could be combined with a sentence. Voluntary forensic treatment is already not uncommon in outpatient settings in Switzerland [11]. However, there would certainly also be a need for inpatient facilities in which voluntary forensic treatment would be possible. Difficulties can arise if the opinion of the person concerned changes; then, the question is, what applies now: ill or not ill? Advance directives, in which the person concerned makes decisions, could help out of the dilemma here.

8.3 Consequences of Preference-Based Psychosocial Support

Preference-based psychosocial support has a certain connection with the question of being ill or not ill, but this is not mandatory. People who declare themselves to be ill may also prefer non-evidence-based interventions. Other people might choose to forgo the medical system from the outset and favour coaching or peer support that is not conventionally covered by health insurance. In addition, various combinations of evidence-based and non-evidence-based programmes are conceivable.

Such development presumably would not have any drastic consequences for the mental state of the general population. The current conventional and evidence-based psychiatric interventions do not leave any traces in terms of "better" health at the population level, as has been empirically established on various occasions [12, 13]. Therefore, even a less evidence-based development should not influence this.

Consequences could possibly be found in the economy. In relevant health economic research, substitution effects are assumed, as has been found for homeopathic and phytotherapeutic products [14]. This means that if certain preference-based medicines are prescribed, this could have a negative effect on other medicines, and vice versa. The key question would then be who finances which services. Would health insurance companies have to agree to everything that their policyholders wanted to use, even if the policyholders did not categorise themselves as "ill"? Or would it be up to the social insurance funds to finance services that the health insurance funds did not want, as these would not be medical services in the strictest sense?

One way out could be a defined catalogue of services that are preferred by people with psychosocial problems. Such a procedure would not be without precedent. Insured persons can already be reimbursed by health insurers for services that are considered preference-based rather than evidence-based. In Switzerland, for example, these services are subject to "medical complementary medicine" [15]. This includes acupuncture, anthroposophic medicine and homeopathy. In Germany, for example, homeopathy is also reimbursable by individual health insurance funds. In the area of social welfare, such catalogues and measures are also not without precedent. As part of the so-called subject financing of services for people with disabilities, which is about to be introduced in the canton of Bern, those affected can choose

both services in defined areas and service providers [16]. However, to counteract a possible increase in volume, people must undergo a needs assessment. A similar instrument exists with the personal budget in Germany [17].

8.4 Harm to Others in a Psychosocial Support Programme Without Coercion

What should happen to people with psychosocial problems who may harm other people or who have actually physically assaulted others? In contrast to self-harm, which is analysed further below, modern society has a system that can, in principle, address all people who are danger to others. Police and judicial institutions are designed to deal with such difficult people and situations. Many people in prison have already been diagnosed as "mentally ill". Psychosis, severe depression and, above all, drug and alcohol problems are widespread in these institutions [8].

Why shouldn't the police and judiciary also be able to deal with people who are today admitted to a psychiatric hospital against their will because they are in danger to others and may be treated there? In my opinion, there is no fundamental argument against this. In addition, when I say in principle, I mean that under the current conditions, people with psychosocial problems are not in good hands in prisons. There is a widespread lack of appropriate support facilities to which people who endanger or have harmed other people are also entitled. This is one of the fundamental dilemmas that needs to be resolved if non-coercive psychosocial support is to be implemented in psychiatric settings.

I am sceptical as to whether this dilemma can be solved within the framework of current institutions for prisoners. We probably need a significantly different system for dealing with punishment in general. I have already mentioned that penalties do not have a major impact on criminal offences but can merely keep people with certain risks away for a certain period of time. Taking this further, a possible way out could be a "quarantine model". Philosophers Gregg Caruso [18] and Derk Pereboom [19] have developed this model in recent years as an alternative to the approach of morally justified punishments, as it is repeatedly advocated, especially in the United States.

The background to the quarantine model is the discussion about free will. Caruso and Pereboom fundamentally question the existence of free will. In contrast, they advocate the approach of "neuroexistentialism" [20]. According to this approach, all actions, decisions and experiences are biologically determined; a soul, a spirit or a corresponding consciousness as a non-physical something does not exist. "The universe is causally closed, and the mind is the brain" ([20]: 8). On the basis of this premise, neuroexistentialism concludes that people bear no moral responsibility for their actions and therefore cannot be punished for their crimes. However, people and societies are allowed to defend themselves against people who pose danger or risk. The quarantine model is then justified by analogy with the fight against epidemics. According to further arguments, it is permissible to keep people away from other people if they pose a danger, even if they are not responsible for this. We have

often experienced this kind of approach with compulsory quarantine during the COVID-19 pandemic. People had to stay indoors after a proven infection to prevent them from passing on the infection.

In my opinion, the quarantine model can generally be applied to people who pose a danger to other people. It is also permissible to keep people with psychosocial problems away from other people if they are in danger to other people, for example, in the event of recurrence and according to relevant prognostic procedures. Moreover, the quarantine model is compatible because the placement must be as least restrictive as possible on the one hand and rehabilitative on the other hand. Caruso calls for rehabilitation to be organised according to the human rights capabilities approach of Martha Nussbaum and Amartya Sen [21]. Capabilities are opportunities for empowerment and realisation, for example, in terms of physical health, social relationships and control over one's own environment. These opportunities for empowerment and realisation are also required in psychiatric rehabilitation and are seen as the background for inclusion and recovery [22].

Furthermore, just as in psychiatric rehabilitation, Caruso calls for basic social conditions to be met, such as the right to housing, healthcare, sufficient income and more. In his view, these conditions are prerequisites for the prevention of criminal offences. In my opinion, they are also conditions for the prevention of some psychosocial problems, such as social exclusion and some mental health problems, in a narrow sense.

Whether and how a quarantine model can be implemented in practice is certainly doubtful under current conditions. However, from the perspective of psychosocial support without coercion outlined here, the model is attractive in various respects. First, there is no need to identify a "disorder" or "illness". Second, the focus is on rehabilitative goals. Third, involuntary therapies must be avoided at all costs. The punitive nature of the institution must be avoided as best as possible.

8.5 Self-Harm in a Psychosocial Support Programme Without Coercion

In contrast to harming others, there is no social institution or social system that could serve as an alternative to a psychiatric hospital in the case of life-threatening self-harm. Therefore, should there be no alternative to compulsory hospitalisation when dealing with suicidal behaviour? The World Health Organisation (WHO) and other UN organisations calling for the abolition of coercion are avoiding a clear answer to this question.

The initial situation regarding the prevention of suicide by coercion can be described as follows [23]: (1) coercive hospitalisations can save the lives of people who would certainly have committed suicide without intervention, (2) false-positive decisions for coercive interventions are unavoidable, as the prediction of suicide is very rarely reliably successful; many people are coercively hospitalised without actually having committed or attempting suicide, (3) the coercive nature of current psychiatric care alone will likely cause people to refrain from seeking support and

possibly commit suicide, and (4) a solution to coercion due to suicidal behaviour that is compatible with the UN CRPD is not apparent. Another complicating factor is that drug treatments [24] and psychosocial interventions also have no clear effects on the prevention of suicidal or self-harming behaviour [25]. The same applies to psychotherapeutic procedures [26] or brief contact interventions after discharge from inpatient treatment [27]. It is currently completely unclear how suicidal or self-harming behaviour can be effectively prevented [28].

These difficulties are also shared from the perspective of affected persons who have experienced suicidal behaviour. However, they make it clear that in conventional psychiatric care, there are virtually no opportunities to report their suicidal ideas or behaviour in a setting that is not associated with restrictive practices [29]. As already mentioned in the introduction, this situation is experienced as an epistemic injustice, and the associated *suicidism* is seen as pejorative discrimination and stigmatisation [30]. In this context, however, we are talking not only about epistemic injustice but also about epistemic violence inflicted on people who want to take their own lives [31].

It is therefore obvious to demand a right to self-chosen death that is not prevented or enabled by medical or disability-related criteria. Any application of medical or other criteria to medically assisted death is criticised in a new article by Alexandre Baril as *sanist* and *ableist* [32]. Baril consistently rejects a medically regulated practice of assisted suicide and instead calls for universal access to assisted suicide options.

Possible ways out of the current practice of attempting to prevent suicide can be derived from these points of view. First, there would need to be an expansion of options for assisted suicide, which would also be accessible to people with psychosocial problems. I myself have proposed making this possible for people who are not in an acute mental health crisis and have a well-considered wish to die [33]. I am convinced that it would be a fundamental task for professionals in the psychosocial support system to accompany people on this path.

Second, there needs to be a support system for people who are suicidal that does not include coercive measures. One approach in this regard is the "Wegloophuis" ("Runaway house") movement developed in the Netherlands in the 1970s [34]. These were facilities run and managed by people with lived experience in psychiatry. In Australia, various types of refuge centres have recently emerged, some of which are also integrated into regular funding. One option is the so-called "subacute recovery and prevention" approach. Its function is both to prevent acute psychiatric treatment and to provide a place that facilitates the transition into the community after acute psychiatric care is provided. A recent study of these facilities revealed that most affected individuals have improved in terms of recovery and that peer counselling in particular is considered very valuable [35].

Another option is a peer-led centre that is responsible for people experiencing suicidal crises. Those affected can stay in this "respite" centre for up to 4 days free of charge and are supported by peers as well as psychological specialists. Reviews have compared such crisis centres with conventional acute psychiatric settings in terms of various indicators and suggest that the crisis centres are rated significantly

better by affected users [36]. However, it cannot be guaranteed on the basis of these studies that the characteristics of the people accompanied were identical.

To my knowledge, whether and to what extent non-psychiatric contact centres for people with suicidal thoughts could prevent such acts has not yet been thoroughly investigated. This would certainly be a worthwhile endeavour. In addition, the relationship between the psychosocial support system and suicidality should be discussed. In my opinion, prevention is not always the top priority. We must accept that people with and without psychosocial problems can see suicide as a legitimate way out of life.

8.6 Dementia and Psychosocial Support Without Coercion

A particular psychosocial problem that can be associated with both self-endangerment and endangerment of others is dementia, which is known to become more likely with increasing age. This problem is usually solved by determining the person's capacity, and in the event of a negative assessment, it is therefore legal and justified according to conventional ethics to carry out measures against the person's will.

Strictly applying the conditions of the UN CRPD is not permissible in principle. The person concerned retains their legal capacity even if they are presumed to be incapable of judgement from an external perspective and may refuse such measures.

One way out of this problem is advance directives. Such documents, which contain the will of the person, are already common in the care of people with dementia, for example, in the question of how to address life-prolonging measures [37]. The handling of coercion in certain situations could also be regulated in advance directives; these regulations could, for example, contain Ulysses clauses that make it possible to act against the will expressed in crisis situations. An advance directive of this kind can either be developed together with the person concerned in the event of the onset of dementia or, as in the case of organ donation in many countries, may even be used across the entire population.

In the case of indicated advance directives, it is necessary under certain circumstances to support them with professionals who have received training in the clinical picture, possible cognitive limitations and appropriate communication skills [38]. However, according to the relevant research, a one-off order is not enough. In a crisis situation, the "now-self" may no longer agree with the decisions of the "then-self" [39]. In this respect, the dispositions should be regularly evaluated with the person concerned and possibly revised. Research is currently being carried out into interaction procedures that go beyond the spoken word to give people with advanced dementia the opportunity to articulate their will [40].

8.7 Conclusions: Towards Psychosocial Support Without Coercion

The situations and dilemmas just described do not make us confident that psychosocial support without coercion could become a reality in the foreseeable future. From a sociological perspective, psychiatry without coercion has considerable consequences for various social systems and institutions. It would require not only a different understanding of "mental illness" but also a fundamentally different psychiatry. Such a fundamentally different psychiatry has recently been outlined by Diana Rose and her husband, sociologist Nicholas Rose. As a person with psychosocial problems herself, Diana Rose has analysed various reform proposals and made them herself. In the end, she and her husband arrive at the following characterisation of a "different psychiatry":

> More fundamentally it would invert the gaze through which distress was to be understood; it would require that all those professionals tried, in whatever way they could, to take the patient's point of view, to try to imagine the world as it is experienced by the patient. A move to epistemic justice demands that the voice of the patient and the experienced reality of the patient, is central to any system of supports and the holistic knowledge of actual lives that this would bring would render the idea of distinct 'components' redundant. Another psychiatry would be one that turns 'patient involvement in research' to 'researcher involvement in patient-led systems. ([41]: 7)

And that would not be enough. We would also need a changed legal and penal system and a drastically changed social law that no longer relies on a diagnosis to fund services. The medicalisation of the legal system is, in fact, one of the major obstacles on the way to abolishing coercion in psychiatry. Another obstacle is the public's positive attitude towards the medicalisation of problems. Linked to this is the widespread belief in the existence and validity of mental illness and psychiatric diagnoses. None of these can be changed quickly.

Until then, however, numerous initiatives could already be launched under the current legal and social conditions. These options range from open ward doors to home treatment, Open Dialogue and supported housing, runaway houses and peer respites, support for suicidal behaviour without coercion in the background, de-escalation and aggression management for all employees and advance directives. These are all topics that can already be realised today without immediately calling for a different psychiatry. The first step would be to consistently implement the ethical demands for the least possible restrictions and coercion as a last resort.

The abolition of coercion in mental health settings will not happen from one day to the next. However, the path should be inevitable.

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